

Competency assessment checklist — ventrogluteal intramuscular administration of testosterone undecanoate 1000 mg/4 mL

Version: 1.1 **Effective date:** 5 October 2025 **Revised:** 2 October 2026 **Next review:** October 2026, or on any change to the Product Information

Prepared by Theodore South, BPharm (Hons), registered pharmacist, Adelaide. Clinics remain responsible for their own procedures, credentialling and emergency arrangements. This document does not replace the Product Information [1], which prevails where they differ.

Used to assess competency in the safe preparation and administration of testosterone undecanoate 1000 mg/4 mL by deep intramuscular injection at the ventrogluteal site. The clinician must demonstrate every criterion under direct observation; see the SOP and training guide for the standard expected.

Clinician details

Name: _____ Position: _____

Assessor: _____ Date: _____

Competency criteria

Criterion	Source	Competent (✓/X)	Comments
Confirms patient identity and date of birth, allergies, contraindications and informed consent, including any prior reaction to an oil-based injection	[1]		
Confirms product, dose (1000 mg in 4 mL), expiry and batch number	[1][2]		
Prepares equipment: 18G drawing-up needle, 20–22G injection needle of suitable length (38 mm; 50 mm if BMI over 30), 5 mL syringe, antiseptic, dressing, sharps container	[5][10]		
Confirms emergency equipment is available: oxygen, adrenaline, monitoring, means of calling 000	[8]		
Positions the patient and landmarks the	[4][5]		

ventrogluteal site correctly (greater trochanter, ASIS, iliac crest; centre of the V)			
Inserts the needle at 90° to a depth that reaches muscle; aspirates for blood return	[5]		
Injects the full 4 mL very slowly, over approximately 2 minutes	[1][3][10]		
Withdraws the needle, applies gentle pressure without massage, applies dressing, disposes of sharps immediately	[5]		
Observes the patient during and for at least 15 minutes after the injection	[1][3]		
Describes the signs of POME and of anaphylaxis, and the immediate management of each, including adult adrenaline dose and route	[6][7][8]		
Documents product, batch, expiry, dose, side, needle, clinician, observation outcome and next due date; knows how to report an adverse event to the TGA	[1][9]		

Assessment outcome

- ☐ Competent
- ☐ Requires further supervised practice
- ☐ Not yet competent

Assessor signature: _____ Date: _____

References

[1] Aspen Pharmacare Australia. Reandron 1000 (testosterone undecanoate 1000 mg/4 mL) solution for injection — Australian Product Information and Consumer Medicine Information. Accessed via the Medicine

Finder, Australian Commission on Safety and Quality in Health Care, 1 October 2026.

<https://www.safetyandquality.gov.au/medicine-finder/reandron-1000-solution-for-injection>

[2] Therapeutic Goods Administration. REANDRON 1000 testosterone undecanoate 1000 mg/4 mL injection vial — ARTG 213942. Canberra: TGA. <https://www.tga.gov.au/resources/artg/213942>

[3] Bayer New Zealand. Reandron 1000 (1000 mg/4 mL solution for injection) — New Zealand Data Sheet. Wellington: Medsafe. <https://medsafe.govt.nz/profs/Datasheet/r/Reandron1000inj.pdf>

[4] SA Health. Changes to your long acting injection — patient brochure on the ventrogluteal site (includes the landmarking figure). Adelaide: Department for Health and Wellbeing, Government of South Australia, 2022. [https://www.sahealth.sa.gov.au/wps/wcm/connect/e185e690-bc60-402f-b264-5ed189fd0390/Long+Acting+Intramuscular+Injections+Brochure+\(1\).pdf?MOD=AJPERES](https://www.sahealth.sa.gov.au/wps/wcm/connect/e185e690-bc60-402f-b264-5ed189fd0390/Long+Acting+Intramuscular+Injections+Brochure+(1).pdf?MOD=AJPERES)

[5] South Eastern Sydney Local Health District. Intramuscular injection in mental health — guideline SESLHDGL/076, version 2.6, 20 January 2025. Sydney: NSW Health. <https://www.seslhd.health.nsw.gov.au/sites/default/files/documents/SESLHDGL%20076%20-%20Intramuscular%20Injection%20in%20Mental%20Health.pdf>

[6] Pastuszak AW, Hu Y, Freid JD. Occurrence of pulmonary oil microembolism after testosterone undecanoate injection: a postmarketing safety analysis. *Sex Med.* 2020;8(2):237–242. doi:10.1016/j.esxm.2020.01.009 <https://doi.org/10.1016/j.esxm.2020.01.009>

[7] Adam JA, Pastuszak AW, Christensen MB, Spencer R, Sandberg A, Hotaling JM, Lipshultz LI. Occurrence of pulmonary oil microembolism (POME) with intramuscular testosterone undecanoate injection: literature review. *Int J Impot Res.* 2023;35(5):439–446. doi:10.1038/s41443-022-00585-1 <https://doi.org/10.1038/s41443-022-00585-1>

[8] Australasian Society of Clinical Immunology and Allergy. ASCIA guidelines: acute management of anaphylaxis. July 2026. https://www.allergy.org.au/images/ASCIA_HP_Guidelines_Acute_Management_Anaphylaxis_July_2026.pdf

[9] Therapeutic Goods Administration. Reporting adverse events — medicines and vaccines. Canberra: TGA. <https://www.tga.gov.au/reporting-problems>

[10] Laboratorios Liconsa. Guidance on the administration of testosterone undecanoate 1000 mg/4 mL — risk minimisation material approved by the MHRA, version 2.0, July 2024. (Same formulation as Reandron 1000.) <https://www.medicines.org.uk/emc/rmm/100105/Document>